TRAUMA-02 | For therapists | Two separate age-based pathways

Assessment, suitability and a shared plan

1 | Purpose, fit and boundaries

Purpose of this unit

Organize goals, needs and preferences before assessment and treatment selection. Professional assessment is separate and includes functioning, environment, resources and current protection needs.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

Do not diagnose PTSD, rate severity or infer model suitability from answers. Do not demand an event account. Possible ongoing harm needs professional assessment and protection; do not call it merely a past reminder.

Reusing existing material

TRAUMA-01 supports choice before a conversation. Unit 2 organizes decisions after assessment; existing INT and SES provide a framework without replacing specialized assessment.

A fictional example for discussion

Fictional example: Itay wants to study comfortably again but does not want to write details. The therapist explores current difficulties and assessment needs with him and a suitable supporter before choosing an approach.

> Draft for administrator review. Clinical review and final rights approval are still pending.

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Working together with the worksheet

2 | Age-based pathways

CH / TE | Support within a TF-CBT pathway

Explore needs and functioning with the child or teenager and a suitable, safe supporter. W brings the young person's voice into planning; it is not a trauma interview or screening questionnaire.

AD / OA | Support within a CPT pathway

Explore current difficulty and meaning, explain the proposed pathway and assess preferences. W does not diagnose or establish CPT suitability; selecting the framework and measures is separate professional work.

Parallel work with a suitable support person

From the start, assess who can participate without endangering safety or privacy. Explore their own needs and possible support. Family relationship alone does not establish suitability.

Using W and P sheets

Choose the age version and relevant questions; completing every sheet is not a condition for progress. W accompanies professional conversation. P records only pre-agreed follow-up. Unit number does not mean session number.

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

Possible difficulties

Difficulty sharing does not prove noncooperation. Explore access, language, privacy and current circumstances; help must not depend on a detailed story or every field being completed.

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Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

CH | Children, approximately ages 6 to 11

CH uses simple words for the child's voice alongside therapist assessment and suitable supporter information. Adults do not supply the child's opinions. Assess developmental understanding, not just chronological age.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

TE | Adolescents, approximately ages 12 to 17

TE explores independent goals, studies, relationships and sharing limits. Ages 15–17 need appropriate review; a younger-child workbook does not establish suitability for them.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

AD | Adults

AD selects CPT after assessment and explanation of options. Record preferences and goals without promising fit or outcomes from this sheet.

OA | Older adults

OA considers life history, health and access individually. Do not automatically attribute new difficulty to trauma or assume cognitive decline or need for accompaniment because of age.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

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Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

P records support and learning agreed in treatment. It sets no exposure, trauma narrative, dosage or component sequence. Without a plan, bring questions to the session. CH/TE sheets do not replace separate supporter work; AD/OA sheets are not official CPT materials.

At the next session

Review changed needs, consent and conditions. Update the plan with the person. Goals, assessment methods and monitoring frequency are set separately, not by counting forms.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

Treatment-manual mapping status

Proposed: TF-CBT for children/teens; CPT for adults/older adults. Full manuals remain unmapped. Age, rights, supporter materials and separate-guide needs await review. Original support drafts, not an approved protocol.

www.nctsn.org/interventions/trauma-focused-cognitive-behavioral-therapy
tfcbt.org/about/
www.ptsd.va.gov/professional/treat/txessentials/cpt_for_ptsd_pro.asp

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